All staff should be aware of fire safety and the steps to follow if a fire should occur. Health care staff should be able to locate the fire extinguishers and know how to use them. Know where the nearest emergency exits are located and understand the evacuation plan if the fire is too large or begins spreading. To remember the order of action in case of a fire, remember the acronym **RACE**.

- **R:** Rescue others who are in danger.
- **A:** Alarm: Activate the fire alarm.
- **C:** Confine the fire by closing doors and windows.
- **E:** Extinguish the fire with the nearest fire extinguisher.

First Aid and CPR

For phlebotomists, training in first aid and CPR is crucial. These skills can be critical in emergency situations that may arise during the blood collection process. Phlebotomists frequently work with patients who may experience nausea, dizziness, seizures, or syncope during blood collections and other procedures. They may also experience conditions caused by the procedure, such as hematoma or excessive bleeding.

When these events occur, knowing how to provide immediate first aid, including elevating the legs of the patient or applying cold compresses, can prevent further issues or complications. While less likely, phlebotomists may be near or with a patient when they experience one of many different types of emergencies. They must know the first aid measures required for each type of emergency. With all emergencies, stay with the patient or confirm that someone is with them until they have recovered or been transported for treatment.

2.7 CHALLENGE

1. Place the steps for actions to take in a fire in the correct order.
   A. Confine the fire by closing doors and windows.
   B. Activate the fire alarm.
   C. Rescue others who are in danger.
   D. Extinguish the fire with the nearest fire extinguisher.

2.7 CHALLENGE KEY

1. The correct order is:
   1.) Rescue others who are in danger. This step comes first as this is the first step that should be taken when a fire occurs, rescuing other people who are in danger, if able.
   2.) Activate the fire alarm. The fire alarm should be activated as soon as possible to alert everyone that there is a fire. That immediate action must be taken to evacuate or assist in the following steps.
   3.) Confine the fire by closing doors and windows. When able, all doors and windows should be closed. This helps with trying to contain the fire to one area and lessen or slow the spread of the fire.
   4.) Extinguish the fire with the nearest fire extinguisher. If the fire is contained and safe to do so, the fire should be extinguished using the nearest fire extinguisher available.

TABLE 2.2 Emergencies

TYPE OF EMERGENCY	SYMPTOMS	FIRST AID
External hemorrhage	Abnormal or profuse bleeding	Apply pressure using gauze or cloth.
Shock	Pale, cold, clammy skin, rapid pulse, fast and shallow breathing	Elevate legs so head is lower than rest of the body, keep patient lying down.
Stroke	Sudden numbness or weakness in face, arm, or leg on one side of the body, sudden confusion or trouble speaking, sudden trouble seeing or walking, sudden severe headache	Recognize signs and call 911 or alert provider immediately.
Cardiac event	Patient is unresponsive, no pulse, not breathing	Begin CPR and use an AED when required.
Nausea	Patients may state that they do not feel good or may vomit. Greenish hue to the skin, sweating, and deep gulping or excessive saliva	Stop procedure. Provide an emesis basin. Provide privacy and a wet cold compress. Do not provide food or drink until the patient recovers.
Seizure	Suddenly becoming rigid and making jerking motions. Patients may appear conscious but unresponsive in the case of a petit mal. A grand mal can lead to full body convulsions.	Stop procedure. Call for help. Guide the patient to the floor and place on side when possible. Do not attempt to restrain or hold a person experiencing a seizure.
Syncope	Sweating, pale or flush, shallow breathing	Stop procedure. Lean close to the patient to help reduce the risk of injury. Loosen tight clothing. Place a cold compress on their neck or head. Maintain patient in reclined position until recovered.
Hematoma	A swelling that gathers quickly at the site of the needle insertion. May be discolored.	Stop the procedure and provide pressure on the site for at least 5 minutes to ensure the bleeding stops.
Accidental artery puncture	Blood will be cherry red and filling with force into the ETS tube.	Withdraw needle immediately. Place pressure on the site for 5 to 15 min.

TABLE 2.2 Emergencies *(continued)*

TYPE OF EMERGENCY	SYMPTOMS	FIRST AID
Accidental needle stick	Feeling a sudden sharp pain near the needle and/or blood in the glove	Remove needle if stuck in skin. Rinse the area thoroughly. Document the incident. Seek medical care within 24 hr. May be necessary for testing for diseases such as HIV and hepatitis C and B.
Petechiae	Small red spots appearing on the arm surrounding the tourniquet	Continue the collection, apply adequate pressure afterward to help stop bleeding promptly.

CPR training is vital for any health care team member in the unlikely event that a patient experiences cardiac arrest or stops breathing at any time during their time in the hospital or medical office. Knowing how to perform chest compressions, use an AED, or assist in maintaining an open airway, allows the phlebotomist to provide life-saving interventions until more advanced medical help arrives to take over. Certification in CPR and first aid ensures that the phlebotomist follows workplace safety regulations or standards and keeps them up to date on best practices for both.

It is important to know the proper CPR steps.

For an adult needing CPR:

1. Check the scene for safety and use PPE.
2. Check for responsiveness, breathing, and life-threatening bleeding.
3. Call or direct an individual to call 911.
4. Gather or direct an individual to get an AED.
5. Ensure the person is on their back on a firm, flat surface and kneel beside them.
6. Provide 30 chest compressions.
7. Administer two rescue breaths.
8. Continue giving sets of 30 chest compressions and two breaths. Use the AED as soon as one is available. Follow the prompts on the AED. Continue until signs of life are seen or help arrives.

Warning Signs of a Stroke

Memorize the acronym **FAST** to remember the warning signs of a stroke.

F = Face drooping

A = Arm weakness

S = Speech difficulty

T = Time to call 911

For a child or infant needing CPR:

1. Check the scene for safety and use PPE.
2. Check for responsiveness by shouting to get the baby or child's attention. For a child, tap their shoulder and shout, and for a baby, tap their foot and shout, while checking for breathing and life-threatening bleeding.
3. Call or direct someone to call 911.
4. Gather or direct someone to get the AED.
5. Ensure the child or baby is on their back on a firm, flat surface. For a child, kneel beside them. For a baby, stand or kneel to the side of the baby with hips at a slight angle.
6. Give 30 compressions. For a small child, use one hand in the center of the child's chest. For a baby, place both thumbs on the center of the chest.
7. Administer two rescue breaths.
8. Continue with sets of 30 compressions and two breaths until an AED is ready to use. Continue until signs of life are seen or another responder is available to take over.

Documentation and Reporting Requirements

Accurate documentation and reporting are important parts of a phlebotomist's job, ensuring proper patient care, compliance, and integrity of laboratory results. Documentation is also important for patient safety and can be used for legal purposes in a court of law. Any documentation done in the patient's medical record must be factual and objective. Proper documentation can help prevent errors, which can lead to potential harm to the patient. Reporting is another part of the phlebotomist's job. This may involve reporting and documenting an adverse reaction during or after the blood collection process or reporting incidents internally.

Incidents that may need reporting include accidental needlesticks and exposure to bloodborne pathogens or other hazardous materials. When reporting must occur, an incident report must be filled out promptly. These forms are confidential and include what occurred, actions taken, and any corrective actions that may need to be taken. If a report involves a patient, the incident should be reported to the appropriate health care team members to ensure prompt and effective follow-up.